

Title

Treatment Sequencing and Subgroup Response Patterns in Systemic Psoriasis Therapy

Abstract

Secondary analyses examined treatment response patterns, demographic predictors, and outcomes of sequential therapy in our 352-participant trial. Baseline factors associated with favorable IL-23 inhibitor response included lower BMI (mean BMI: responders 26.1 vs nonresponders 28.7, $p = 0.03$) and absence of joint symptoms (skin-only psoriasis 61% response vs psoriatic arthritis 48% response). Participants achieving complete response by week 16 showed sustained clearing through 52 weeks (92% maintained $\geq 90\%$ clearance). Among 83 participants who switched from oral to IL-23 inhibitor due to insufficient response, 72% achieved major improvement at subsequent assessment, demonstrating salvage efficacy. Gender differences in response were minimal, though women reported greater quality-of-life gains from treatment. Cost-effectiveness analysis suggested IL-23 inhibitor would be cost-effective if initial efficacy advantage translates to reduced long-term complications. Systemic safety events were rare in both groups over 52 weeks; immunogenicity against IL-23 inhibitor developed in 3% of treated patients. Real-world adherence was higher with IL-23 inhibitor (83% at 52 weeks vs 71% with oral agent). These secondary analyses support IL-23 inhibitor as a preferred first-line option for systemic-eligible patients, with oral agents retaining utility in cost-constrained settings or specific subgroups.